

NEXUS CLINICAL LABORATORIES

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LABORATORY REPORT

Patient: Ethan Johnson	DOB: July 02, 1993 (Age 53)
MRN: MRN-9683481	Gender: Female
Ordering Physician: Dr. Angela Forsythe, MD — Cardiology	Accession #: NX-20055070
Specimen Type: Venous Blood / Urine	Collected: April 09, 2025 at 12:00 AM
Received: April 05, 2025 at 02:00 AM	Reported: May 13, 2025 at 05:00 AM

COMPLETE BLOOD COUNT (CBC) WITH DIFFERENTIAL

Test	Result	Units	Reference Range	Flag
WBC — White Blood Cell Count	4.7	x10 ³ /μL	4.5 – 11.0	
RBC — Red Blood Cell Count	4.2	x10 ⁶ /μL	3.80 – 5.10	
Hemoglobin	14.5	g/dL	12.0 – 16.0	
Hematocrit	35.4	%	36.0 – 46.0	L
MCV — Mean Corpuscular Volume	72.0	fL	80.0 – 100.0	L
MCH	33.2	pg	27.0 – 33.0	
Platelets	432	x10 ³ /μL	150 – 400	H
Neutrophils	66.3	%	50.0 – 70.0	
Lymphocytes	19.3	%	20.0 – 40.0	L

COMPREHENSIVE METABOLIC PANEL (CMP)

Test	Result	Units	Reference Range	Flag
Glucose (Fasting)	105	mg/dL	70 – 99	H
BUN — Blood Urea Nitrogen	21	mg/dL	7 – 25	
Creatinine	0.73	mg/dL	0.50 – 1.10	
eGFR	71	mL/min	> 60	
Sodium	145	mEq/L	136 – 145	
Potassium	4.1	mEq/L	3.5 – 5.0	
Calcium	9.2	mg/dL	8.5 – 10.2	
AST	41	U/L	10 – 40	
ALT	54	U/L	7 – 56	
Total Bilirubin	0.3	mg/dL	0.1 – 1.2	
Albumin	4.4	g/dL	3.5 – 5.0	

THYROID FUNCTION PANEL

Test	Result	Units	Reference Range	Flag
TSH — Thyroid Stimulating Hormone	4.95	mIU/L	0.45 – 4.50	H
Free T4 (FT4)	0.77	ng/dL	0.82 – 1.77	L
Free T3 (FT3)	3.8	pg/mL	2.0 – 4.4	

CLINICAL INTERPRETATION / COMMENTS

Findings are consistent with microcytic hypochromic anemia, likely secondary to iron deficiency; correlation with serum iron studies (ferritin, TIBC) is recommended. Mildly elevated fasting glucose warrants an HbA1c to evaluate for pre-diabetes. TSH elevation with low-normal FT4 is suggestive of subclinical/clinical hypothyroidism; clinical follow-up and repeat thyroid panel in 4–6 weeks is advised.

Reviewed & Verified by: Dr. Linda Ochoa, MD | Pathology & Lab Medicine | May 13, 2025 at 05:00 AM CST | Report ID: NX-20055070